Dr Vikas Deswal, a senior consultant for internal medicine at Medanta, Gurugram, is of a similar opinion: "The XBB.1.16 variant is unlikely to cause severe disease in patients who are vaccinated or who do not have any other comorbidities."

In fact, while the constantly mutating Omicron may continue to give rise to new sub-variants that can infect better, they are unlikely to be as severe as the Delta variant, which killed lakhs of people in 2021, says Rakesh Mishra, director of the Tata Institute for Genetics and Society. "The virus mutates to survive. Over time, it'll start focusing on mutations that allow it to infect and replicate continuously. Causing more severe symptoms is actually not in the best interest of the virus's survival plan."

Thus, virologists and doctors aren't overly concerned for now. In the capital, for instance, most hospital beds remain empty, even though five districts have reported a TPR of over 10 per cent—South (15.8%), East (15.2%), North-East (13.6%), Central (10.4%) and South-East (10.2%). As of March 27, there were only two patients in the ward of the Lok Nayak Jai Prakash Narayan (LNJP) Hospital, Delhi's largest Covid-19 facility. "We are not seeing any uptick in the number of patients who need to be admitted. Those who do come in reporting symptoms usually have mild infection," says Dr Suresh Kumar, medical director, LNJP hospital.

The variant is also not causing any new or unusual symptoms, and most people experience only fever, cough, sore throat and runny nose. And so, treatment, too, remains the same. "Mild Covid-19 cases are to be managed at home and care is entirely symptomatic. Patients must self-isolate, stay hydrated, take rest, and use paracetamol for fever and body-aches. There is no role for any antibiotics or antivirals at this stage, and it's only for people who are at high risk of developing complications or severe disease that doctors can consider antiviral medications," says Dr Sandeep Budhiraja, internal medicine specialist and medical director of Max Healthcare.

This, however, does not mean that people can let their guard down. The elderly and those with comorbidities or immunocompromising issues must remain careful, for they are at a greater risk of getting infected as well as developing serious illness. "Covid-appropriate behaviour must be reinforced. Anybody who is symptomatic of disease with flu-like symptoms should isolate and wear a mask, caregivers and other members who are exposed to this person should wear masks, people should avoid crowded places, should wash hands frequently, and follow coughing and sneezing etiquettes," says Dr Budhiraja.

THOUGH THE XBB.1.16 IS CAUSING ONLY MILD SYMPTOMS IN MOST PEOPLE, THE ELDERLY AND THOSE WITH COMORBIDITIES MUST BE CAREFUL

It is also important to understand that prior infection and being fully vaccinated do not mean that one won't get infected again. "The risk of infection is still there, but the severity of disease is less. There is no need to take an extra dose of a Covid-19 vaccine as of now, but those who have not yet taken their three doses must get vaccinated," says Dr Arora.

What experts mean is that though things are currently under control, India is not completely out of the woods yet. Letting the virus run wild and unchecked through the population not only poses a threat to vulnerable groups but also raises the chances of new variants coming up. And with a virus that has proven to be as unpredictable as the SARS-CoV-2, it's best not to take chances. ■

By Ajay Sukumaran

Just the other day, Basavaraj Bommai had made a conspicuous break from convention. After he convened a full cabinet meet on March 24, Karnataka's chief minister chose to brief the press himself, instead of deputing a senior minister. The proactive urgency explained itself within days, as May 10 was declared as the date for assembly polls. In effect, that was his last pre-poll cabinet meet—and the agenda, brimful with 119 items, indeed matched up to the occasion. Two moves, in particular, were loaded—the allocation of internal quotas for various subgroups in the Scheduled Castes (SC) list, and the increase in Other Backward Classes (OBC) reservation for the two key caste groups in the state, Veerashaiva-Lingayats and Vokkaligas. Especially how the space for the latter was created.

Taking up these matters, the CM noted, had been akin to 'stirring up a hornet's nest'. "For 2-3 decades, governments kept putting things off. There had been no effort to find solutions," he said. But the solution that the BJP came up with was essentially to take Muslims out of backward classes reservation, thereby freeing up a 4 per cent quota which would then be equally redistributed among the Lingayats and Vokkaligas.

The move immediately came in for sharp criticism from various quarters and the two main opposition parties, the Congress and Janata Dal (Secular), which called it an election gimmick to polarise communities. It was the second time in four months that the Bommai government was resorting to a rejig of OBC reservations—in December, amid agitations by the Panchamashali sub-sect of the Lingayat community seeking a 15 per cent quota, the government had created two new slabs to accommodate both Lingayats and Vokkaligas, assuring that they could claim the spillover surplus from the 10 per cent reservation for Economically Weaker Sections (EWS).

Now, the government has changed tack and proposes to shift Muslims—who come under Category II(b) with 4 per cent reservation—to the EWS group, reasoning that there is no constitutional provision for reservation for religious minorities. Bommai described it as